Chapter 20: Putting the TOGAF Standard into Practice

Study Guide, based on Mastering the TOGAF Standard by Eric Jager (2025), Chapter 20 (pp. 781-815)

Overview

Chapter 20 (pp. 781-815) is a single end-to-end case that shows the whole book working together. Jim, the Enterprise Architect of the hospital **Academic Diagnostic Medicine**, turns a 2028-2030 strategy into an implemented architecture using a tailored ADM (p. 781). Section 20.1 follows him through seven time-boxed stages, from March 2027 to August 2030: scoping the work, envisioning, starting development, setting direction, getting ready for implementation, overseeing, and maintaining control. Each stage produces specific artifacts (pp. 782-808). Section 20.2 then maps the stages back to ADM phases and the steps actually used (pp. 808-815). Leave with this idea: Jim used only the ADM steps that added value, iterated and jumped between phases, and linked every requirement, work package, and roadmap item to goals. That is TOGAF "as intended" (p. 815). Chapter 20 is part of the GREEN and GOLD reading tours (p. 781).

> **Reading note:** The whole chapter is Jager's fictional case, not Standard text. It reuses artifacts and techniques from Chapters 5, 7, 12, and 14 (Driver/Goal/Objective Diagram, Requirements Catalog, Stakeholder Catalog, Plateau Planning, Work Package Portfolio Map, Implementation Factor Catalog, Business Value and Risk Matrix). It is the best chapter for seeing how the parts connect. Note the tailoring choices: Jim *skipped* a maturity assessment and *added* a risk entity and Risk Catalog (p. 809).

Detailed Section Breakdown

Chapter Opener (p. 781)

- TOGAF richly describes metamodel and Content Framework tailoring, the repository, governance, and techniques, but applying them is a challenge. This case shows it done (p. 781).

20.1 A Case Scenario (pp. 782-808)

- **Setting.** A patient-centred hospital with a 2028-2030 agenda: patient satisfaction, workflow optimization, financial health. It has fairly high architecture maturity and TOGAF has long been its standard. Jim's role is to translate drivers into capabilities, align business and IT, and keep change structured, traceable, and value-driven (p. 782).

Stage	Period	What Jim did	Key artifacts	Page
20.1.1 Scoping the Work	Mar-Apr 2027	Two board strategy meetings. Visualized the strategy and proposed three projects (improve patient satisfaction, improve clinical workflows, reduce operating costs). The board approved and gave requirements. Planned to add a *risk* entity to the metamodel	Driver/Goal/Objective Diagram (Fig. 20-1); Requirements Catalog linked to goals (Table 20-1)	pp. 782-784

Stage	Period	What Jim did	Key artifacts	Page
20.1.2 Envisioning the Architecture	May-Jun 2027	Interviewed board, executives, clinical and operational staff. Classified stakeholders. Recorded what communication worked. Captured concerns and constraints (e.g., low digital literacy of older patients, resistance to standardization, retraining, speed of savings)	Stakeholder Catalog (Table 20-2); Communications Plan; concerns and constraints (Table 20-3); Statement of Architecture Work	pp. 784-787
20.1.3 Starting the Development	Jul-Oct 2027	Reused repository building blocks. Linked strategy to capabilities. Got the Statement of Architecture Work approved. Built baselines and found missing capabilities (Feedback Analysis, Supply Chain Management), processes, and roles (Patient Safety Officer, Financial Analyst). Heat-mapped capabilities. Baseline and target application views. Accepted the COO's no-infrastructure-change constraint (cloud already in place). First ADD	Business Capability Map (Tables 20-4, 20-6); Capability/Business Process Matrix (Tables 20-5, 20-7); Strategy/Capability Diagram (Fig. 20-2); Organization/Business Roles Catalog (Table 20-8); Capability Heat Map (Fig. 20-3); Process/Application Realization Diagrams (Figs. 20-4, 20-5)	pp. 787-793
20.1.4 Setting the Direction	Nov-Dec 2027	Two Transition Architectures. Gaps A, B, C closed by work packages linked to goals. Draft roadmap with work packages colour-coded by project. HR to fill missing roles. Checked impact on running projects (none). Stakeholder sessions. Requirements agreed as criteria. Rationale documented. ADD finalized after two feedback iterations	Plateau Planning Diagram (Fig. 20-6); Work Package Portfolio Map (Table 20-9); Draft Architecture Roadmap (Fig. 20-7); ADD	pp. 794-798
20.1.5 Getting Ready for Implementation	Jan-May 2028	Identified three implementation factors (automating discharge planning, training the Planner role, feedback analysis distinguishing positive and negative). Revisited concerns and constraints. Three-phase migration plan respecting dependencies (new capabilities first). Readiness assessment flagged employee engagement and skills (score 3). Plan approved. Roles filled. Project manager adopts the plan. Risks plotted per project and per building block. Formal approval	Implementation Factor Catalog (Table 20-10); Implementation and Migration Plan (Table 20-11); BTRA (Table 20-12); Business Value and Risk Matrix (Fig. 20-8); Risk Catalog; Risk/Process/Application Realization Diagram (Fig. 20-9)	pp. 798-805
20.1.6 Overseeing the Architecture Work	Jun 2028-Aug 2029	Work carried out under the Architecture Contract. Random compliance checks, with reports shared. Transition A delivered. Capability map and heat map updated (new capabilities working, Patient Feedback Management improved)	Architecture Contract; Architecture Compliance Report; updated Capability Heat Map (Fig. 20-10)	pp. 805-806

Stage	Period	What Jim did	Key artifacts	Page
20.1.7 Maintaining Control	Sep 2029-Aug 2030	Stakeholder questionnaires and performance reviews to manage change. Heat-mapped the strategy for the board. Investigated lagging objectives: the Planning Application's learning curve delayed the throughput target, and the cost focus had been on recording before reducing. Target Architecture nearly reached	Driver/Goal/Objective Diagram Heat Map (Fig. 20-11)	pp. 806-808

- **Why linking matters.** Requirements tied to goals keep them clear to everyone (p. 784). Work packages mapped to objectives' timeframes make progress traceable (pp. 795-796).
- **Risk becomes first-class.** By adding a risk entity early, Jim could capture risks in a Risk Catalog and later visualize them on projects (Business Value and Risk Matrix: optimize clinical workflows is the highest risk because of automation and retraining) and on building blocks (traffic-light scores across five levels) (pp. 784, 799-805).

20.2 Summary (pp. 808-815)

- Jim covered all ADM phases, iterating and sometimes jumping ahead and back, which the Standard's adaptability allows (p. 808).

Period	ADM phase(s)	Steps actually performed	Page
Mar-Apr 2027	Preliminary	Identify key drivers and context; assess EA maturity (skipped in practice, since already mature); choose and tailor the framework (added the risk entity and Risk Catalog)	p. 809
May-Jun 2027	A	Establish the project; identify stakeholders and requirements; high-level vision (Strategy/Capability Diagram); Statement of Architecture Work approved	p. 810
Jul-Oct 2027	B, C, D	Select models and tools (repository reuse); Baseline and Target; gap analysis; candidate roadmap components; stakeholder reviews; ADD	pp. 811-812
Nov-Dec 2027	E	Corporate change attributes (Implementation Factor Catalog); interoperability; group work packages; Transition Architectures; roadmap and migration plan	pp. 812-813
Jan-May 2028	F	Align with project management; assign business value (Business Value and Risk Matrix); prioritize; confirm roadmap; complete plan	pp. 813-814
Jun 2028-Aug 2029	G	Confirm scope and priorities (Work Package Portfolio Map); compliance reviews; post-implementation review after each transition	pp. 814-815
Sep 2029-Jul 2030	H	Monitoring tools (dashboards); manage risks; analysis (strategy heat map). New risks feed the next ADM iteration	p. 815

- **The lesson.** "Not all of the method's steps were necessary... he only used the parts that added value... He used the TOGAF Standard as intended" (p. 815).

Key Terms & Definitions

Term	Definition (as used in the case)	Page
Strategy/Capability Diagram	Combines the Driver/Goal/Objective Diagram with the Business Capability Map to show where strategy hits capabilities	p. 788
Capability/Business Process Matrix	A tailored artifact relating capabilities to existing (or missing) processes	pp. 787-790

Term	Definition (as used in the case)	Page
Organization/Business Roles Catalog	Roles per unit with status (existing or non-existent)	p. 790
Risk Catalog	A tailored artifact made possible by adding a risk entity to the metamodel	pp. 784, 804
Risk/Process/Application Realization Diagram	Risks attached to building blocks with traffic-light scores	p. 805
Driver/Goal/Objective Heat Map	Strategy diagram coloured by achievement, used for board reporting	p. 807

Figures & Tables Referenced

Figure / Table	What it shows	Page
Figure 20-1, Table 20-1	The hospital's strategy. Requirements Catalog	pp. 783-784
Tables 20-2, 20-3	Stakeholder Catalog. Concerns and constraints	pp. 785-786
Tables 20-4 to 20-8, Figure 20-2	Capability map, capability/process matrix, missing capabilities, processes, roles. Strategy/Capability Diagram	pp. 787-790
Figures 20-3 to 20-5	Capability Heat Map. Baseline and target Process/Application Realization	pp. 791-793
Figure 20-6, Table 20-9, Figure 20-7	Plateau Planning. Work Package Portfolio Map. Draft roadmap	pp. 794-796
Tables 20-10 to 20-12	Implementation Factor Catalog. Migration plan. Readiness assessment	pp. 799-802
Figures 20-8, 20-9	Business Value and Risk Matrix. Risk/Process/Application Realization	pp. 804-805
Figures 20-10, 20-11	Updated Capability Heat Map. Driver/Goal/Objective Heat Map	pp. 806-807
Tables 20-13 to 20-19	Steps performed per ADM phase	pp. 809-815

Self-Check Questions

Q: What artifact did Jim use first to visualize the strategy, and what did it lead to?

A: A Driver/Goal/Objective Diagram, which suggested three projects: improve patient satisfaction, improve clinical workflows, and reduce operating costs (pp. 782-783).

Q: Why did Jim link each requirement to a goal?

A: So that the requirements for achieving each goal were clear to everyone at all times (p. 784).

Q: Which tailoring decisions did Jim make in the Preliminary Phase?

A: He skipped the maturity measurement (the organization was already mature with TOGAF) and added a risk entity to the metamodel and a Risk Catalog to the Content Framework (p. 809).

Q: What gaps did the baseline analysis reveal?

A: Missing capabilities (Feedback Analysis, Supply Chain Management), missing processes (feedback trend analysis, infection prevention audit, supply chain rationalization), and missing roles (Patient Safety Officer, Financial Analyst) (pp. 789-790).

Q: How did Jim handle the COO's constraint on infrastructure?

A: He adapted the architecture to the existing cloud infrastructure, since there was no budget for changes (p. 793).

Q: Which readiness factors needed attention, and what was the remedy?

A: Employee engagement and skills and competencies (both scored 3), linked to automating discharge planning. The remedy was training staff (pp. 802-803).

Q: Which project carried the highest risk on the Business Value and Risk Matrix, and why?

A: Optimize clinical workflows, because it automates manual tasks and requires the Planner role to be reinterpreted and retrained (p. 804).

Q: Why was the 10% throughput objective late?

A: The new Planning Application had a steeper learning curve than assumed, which delayed adoption (p. 807).

Q: In which ADM phase did Jim use the Work Package Portfolio Map to prioritize execution?

A: Phase G, to confirm scope and priorities and keep activities in the right order (p. 814).

Q: What is the overall lesson of the case?

A: Tailor the ADM: use only the steps that add value, iterate as needed, and keep everything traceable to goals. That is how TOGAF is intended to be used (pp. 808, 815).